Nursing Essentials

Priority: Activate the hemorrhage response for concerning bleeding or instability. Do not wait for a blood-loss threshold before acting.

FOCUS 1

Recognize

- **Definition anchor:** cumulative loss of at least 1,000 mL, or bleeding with hypovolemia signs within 24 hours of birth, regardless of route.
- **Earlier action:** lower volumes can still be abnormal and require intervention, especially with symptoms or continued loss.
- **Quantify:** measured collection and weighed materials; reassess perfusion, vital signs, mental status, and urine output.

FOCUS 2

Find the cause

- **Tone:** uterine atony—often a soft / boggy uterus.
- **Trauma:** laceration, hematoma, or rupture; persistent bleeding can occur despite a firm uterus.
- **Tissue:** retained placental material. **Thrombin:** impaired coagulation. More than one cause may coexist.

FOCUS 3

Act together

- **Call / activate:** obtain assistance and follow the hemorrhage protocol; assess the likely cause while supporting circulation.
- **If atony:** fundal massage, bladder management, and ordered uterotonics; continue reassessment.
- **Prepare concurrently:** adequate IV access, blood tests, blood products, warming, and further procedures as indicated.

Medication safety

Make it stick: A firm uterus does not end the assessment. Ongoing bleeding still requires evaluation for trauma, retained tissue, and coagulation problems.

Focus	What to know	Nursing application
Oxytocin	commonly first-line for atony.	Assess tone and response
Methylergonovine	avoid in hypertension / preeclampsia. **Carboprost:** avoid in asthma.	Screen contraindications
Tranexamic acid	administer promptly when indicated under the protocol; the recommended early-treatment window is within 3 hours of birth.	Follow the hemorrhage pathway

RED FLAGS · ESCALATE PROMPTLY

Activate the hemorrhage response for ongoing bleeding or poor perfusion. Do not wait for a blood-loss threshold or a delayed blood-pressure drop.